February 9, 2026

RE: CMS Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

The Cooperative Exchange, the National Clearinghouse Association is composed of 23 member organizations[1], representing over 90% of the clearinghouse industry that supports over 1 million provider organizations, through more than 7,000 payer connections and 1,000 Health Information Technology (HIT) Vendors, and processes over 6 billion transactions annually.

From the perspective of national healthcare clearinghouses that operate the HIPAA-mandated EDI transaction infrastructure supporting clinical and administrative workflows at scale, the primary barriers to AI innovation and adoption are regulatory fragmentation, inconsistent definitions, and the resulting accumulation of technical debt and operational risk accumulation across multiple stakeholder ecosystems.

These stakeholders for healthcare clearinghouses consist of providers, health plans, third-party vendors and multiple jurisdictions and are impacted by differing AI regulatory frameworks in both the private and public sector. The core challenge is that a clearinghouse does not just manage its own AI; it must also navigate the AI data exchange between all stakeholders in the healthcare ecosystem.

In addition, non-standardized regulatory AI mandates create a barrier to implementation limiting the ability to develop, train, and deploy AI models that are scalable at a national level.

The following reflects our comments and recommendations regarding the CMS RFI Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

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[1] The views expressed herein are a compilation of views gathered from our member constituents and reflect the directional feedback of most of its collective members. CE has synthesized member feedback, and the views, opinions, and positions should not be attributed to any single member, and an individual member could disagree with all, or certain views, opinions, and positions expressed by CE.

CMS AI RFI Questions/Responses

Question 1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

- **Fragmented state AI regulatory landscape:** Hundreds of state-level AI statutes, rules, and guidance documents introduce inconsistent definitions, disclosure obligations, and governance expectations. For national clearinghouses processing millions of transactions daily, this fragmentation materially constrains scalable, compliant AI deployment.
- **Structural technical debt from non-standardized mandates:** National clearinghouses face a "logic-forking" crisis where AI models must be localized to accommodate varying state definitions of "automated decision-making". This prevents the training of foundational models on unified national datasets, diluting the predictive power of AI in administrative workflows.
- **Compounding regulatory and liability exposure for national intermediaries:** Clearinghouses operate across all jurisdictions and across payer, provider, and vendor relationships. Misaligned state and federal AI requirements compound legal risk, increase liability ambiguity, and require conservative controls that slow responsible innovation.
- **Operational infeasibility of inconsistent governance at transaction scale:** AI governance requirements that vary by jurisdiction are difficult to operationalize in high-volume automated environments. Manual review, bespoke controls, or state-specific logic cannot scale reliably across eligibility, prior authorization, claims, attachments, and payment workflows.
- **Misalignment between emerging AI mandates and existing federal health IT frameworks:** State AI requirements do not consistently align with established federal privacy, security, interoperability, and transaction standards, creating overlapping and sometimes conflicting compliance obligations for infrastructure operators.
- **Inconsistent AI definitions, transparency, and disclosure obligations:** Divergent state laws and guidance introduce inconsistent definitions, terminology, and thresholds for what constitutes AI use, automation, and decision support, as well as when and how disclosures are required. For national clearinghouses operating high-volume, automated transaction workflows, these inconsistencies make it difficult to encode rules, automate controls, and deliver consistent, accurate disclosures across jurisdictions, even where AI systems are responsibly governed.
- **Unclear accountability across AI value chains:** Ambiguity regarding responsibility among AI developers, payers, providers, and intermediaries increases legal and operational risk, particularly where automated decisions influence access to care, payment timing, or administrative outcomes.

- **Cost and burden of maintaining compliant national infrastructure:** Maintaining secure, auditable, and compliant AI-enabled transaction infrastructure requires significant and ongoing investment. Fragmented regulatory expectations increase cost without corresponding incentives, disproportionately impacting infrastructure-level innovation.

Taken together, these barriers do not merely slow AI adoption—they shape who can deploy AI responsibly at national scale. Greater federal alignment and clarity would reduce unnecessary compliance friction, support trustworthy automation, and enable infrastructure operators to continue advancing AI that improves access, efficiency, and reliability across the healthcare system.

Question 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

- **Make administrative workflow interoperability a first-order AI policy objective:** Accelerate standardization and adoption of interoperable administrative transactions (e.g., eligibility, claim status, prior authorization, attachments, payment) using modern, API-based exchange where feasible. AI effectiveness depends on standardized, reliable inputs across payer, provider, and intermediary ecosystems.
- **Designated Administrative AI Regulatory Sandboxes:**
HHS should prioritize the creation of safe-harbor environments where clearinghouses can pilot AI-driven prior authorizations without immediate liability for "model drift," provided they meet strict federal transparency standards. We need a shift from "process-based" regulation to "outcome-based" benchmarks.
- **Align payment policy incentives to reward infrastructure AI that reduces burden:** Create CMS payment or program signals that encourage adoption of AI-enabled automation that reduces administrative burden, cycle time, and manual intervention. Many high-value AI use cases improve care access and efficiency but are not tied to discrete billable services.
- **Establish clear governance expectations for non-medical device AI used in care operations:** Provide practical governance or safe-harbor frameworks for AI used in operational and administrative workflows that influence care delivery, particularly in high-volume clearinghouse environments. Clearinghouses must manage security, privacy, auditability, and liability across multi-stakeholder ecosystems while enabling AI-driven automation, requiring clear expectations for accountability and risk allocation.

- **Standardize expectations for monitoring, audit trails, and human oversight:** Define scalable, outcome-oriented expectations for post-deployment monitoring, audit trails, and appropriate human oversight in automated environments. For clearinghouses processing millions of transactions across jurisdictions, AI governance must support traceability, security controls, and compliance assurance without undermining operational efficiency or the responsible use of AI at scale.
- **Reduce state-by-state policy fragmentation through federal alignment mechanisms:** Encourage alignment between emerging state AI mandates and existing federal privacy, security, and interoperability frameworks to reduce duplicative compliance burden and support scalable, national AI deployment.
- **Invest in AI-ready benchmarking and test assets for real-world workflows:** Support development of benchmarking datasets, synthetic data, and testing frameworks for AI in real-world clinical and administrative workflows, including measurement of cycle time, accuracy, rework, and downstream impact.

HHS Regulations, Policies, and Programs That Could Be Revisited or Augmented:

- **CMS administrative simplification and transaction modernization initiatives:** Revisit and accelerate CMS efforts related to administrative simplification, prior authorization modernization, attachments, and transaction standards to ensure these programs explicitly support AI-enabled automation, interoperability, and scalable governance across eligibility, claims, and payment workflows.
- **CMS payment policy and value-based care models:** Examine payment policies and alternative payment models where administrative efficiency, automation, and infrastructure-level AI directly affect access, timeliness, and cost of care, but are not explicitly recognized or incentivized.
- **CMS Innovation Center (CMMI) demonstrations:** Leverage CMMI authorities to test and evaluate AI-enabled administrative and operational innovations, including clearinghouse-facilitated solutions that reduce burden, improve throughput, and support care coordination at scale.
- **ONC / ASTP interoperability frameworks and guidance:** Align AI policy development with ONC interoperability frameworks, implementation guidance, and post-certification market expectations to support reliable data access, provenance, and permitted use necessary for responsible AI deployment.
- **Federal privacy, security, and data governance frameworks:** Clarify how existing federal privacy, security, and interoperability frameworks apply to AI-enabled administrative workflows to reduce duplicative or conflicting requirements and support consistent national implementation.

Question 3: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security).

What novel legal and implementation issues exist, and what role, if any, should HHS play to help address them?

Novel legal and implementation issues:

- **Diffuse and overlapping liability across multi-stakeholder AI value chains:** AI-enabled systems and workflows are often deployed across developers, clearinghouses, payers, and providers within shared administrative and care-related processes. Existing legal frameworks do not clearly allocate responsibility when these systems influence access to care, administrative determinations, or payment outcomes, creating uncertainty around liability, indemnification, escalation, and remediation.
- **Challenge of "Algorithmic Forensics" in multi-stakeholder ecosystems.** When an AI-driven transaction fails or is biased, determining whether the fault lies with the data provider, the model developer, or the clearinghouse intermediary is currently impossible under existing HIPAA contracts. HHS must define a **"Chain of Provenance"** standard for AI-influenced metadata.
- **Governance gaps for AI embedded in automated transaction workflows:** AI in clearinghouse environments is often embedded within routing, validation, and exception logic rather than deployed as a discrete tool. This challenges governance models that assume clear system boundaries, identifiable decision points, and manual oversight.
- **Clarifying application of existing privacy and security frameworks and definitions to AI-enabled operations:** AI-enabled administrative and transaction workflows operate within established federal privacy and security frameworks, including HIPAA- and NIST-aligned controls. However, inconsistent state and federal terminology, definitions, and classification of AI use create uncertainty regarding how existing requirements should be applied, evidenced, and audited—particularly for data aggregation, enrichment, monitoring, and permitted use across multi-stakeholder environments. Greater definitional alignment and clarity would support consistent implementation of existing security and privacy controls without introducing duplicative or conflicting obligations.
- **Operationalizing human oversight and accountability at transaction scale:** Many AI governance frameworks appropriately require human accountability, review, and attestation for AI-assisted activities. In high-volume clearinghouse environments, however, this oversight is operationalized through **identity**

validation, role-based authorization, audit trails, exception-based review, and provenance controls, rather than manual review of every transaction. Effective governance depends on standardized mechanisms that enable demonstrable human accountability without disrupting automated transaction flows that support access to care, payment accuracy, and system reliability.

- **Contractual misalignment and indemnification risk:**
  AI-related risk is frequently addressed through bilateral contracts that do not reflect multi-stakeholder transaction ecosystems. This creates gaps where no single stakeholder has end-to-end control over AI behavior, yet multiple parties bear downstream regulatory and legal exposure.

Appropriate role for HHS:

- **Provide role-based governance clarity for non-medical device AI:**
  Articulate expectations that distinguish responsibilities among AI developers, intermediaries (including clearinghouses), payers, and providers, particularly where AI is embedded in shared infrastructure rather than owned or controlled by a single entity.
- **Support scalable, outcome-oriented governance models:**
  Promote governance principles focused on accountability, auditability, and traceability that can be implemented in high-volume, automated environments, rather than prescriptive, tool-specific controls.
- **Align AI governance with standardized, certifiable security and compliance frameworks:**
  AI-enabled administrative and operational workflows should be governed within established federal privacy, security, and interoperability frameworks, including HIPAA- and NIST-aligned controls. HHS should promote **standardized, certifiable governance models** with clear expectations for compliance, audit scope, and evidence, administered by **qualified and certified auditors**. Such an approach supports strong security and accountability while avoiding duplicative or inconsistent audits across jurisdictions and enabling scalable, infrastructure-level AI operations.
- **Encourage consistency across federal and state AI approaches:**
  Use federal leadership to promote alignment between emerging state AI mandates and existing federal health IT and data governance frameworks, particularly for national infrastructure operators.
- **Recognize clearinghouses as governance and audit infrastructure:**
  Acknowledge the role clearinghouses play in operationalizing AI governance, including maintaining audit trails, enabling human-in-the-loop verification, executing disclosures, and supporting payer and provider compliance across multi-stakeholder workflows.

Question 4: For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care?

- Should HHS further support these processes? If so, which mechanisms would be most impactful (*e.g.,* contracts, grants, cooperative agreements, and/or prize competitions)?

From an enterprise compliance and security perspective, effective evaluation of non-medical device AI must be grounded in operational performance, governance, and accountability at scale, rather than narrow model-level accuracy metrics.

Non-medical AI is frequently embedded within automated healthcare workflows that support care delivery and administration, requiring evaluation methods that function reliably across high-volume, multi-stakeholder environments.

Pre-deployment evaluation considerations:
Pre-deployment evaluation should focus on whether AI systems are fit for operational use, including:

- Validation within representative operational workflows, ensuring AI behavior aligns with system controls, logging, escalation, and governance expectations, rather than relying solely on isolated technical testing.
- Data quality, representativeness, and bias assessment, particularly for administrative and operational use cases
- Security and privacy readiness, ensuring alignment with existing HIPAA and NIST-based control frameworks
- Governance clarity, including documentation of system purpose, accountability boundaries, and escalation pathways
- Resilience and Fairness Metrics:
  Evaluation should include "Stress Testing" against synthetic adversarial datasets to ensure models do not fail during high-volume periods (e.g., open enrollment). We should also propose Automated Governance Agents, AI that audits other AI, to provide real-time oversight at transaction scale.

These evaluations should establish baseline expectations for reliability, security, and governance, not clinical efficacy.

Post-deployment monitoring and robustness:
Given the dynamic nature of AI-enabled systems, post-deployment evaluation is critical and should emphasize:

- **Continuous performance monitoring**, including error rates, exception handling, and downstream operational impacts.
- **Auditability and traceability**, enabling reconstruction of AI-influenced outcomes for compliance, dispute resolution, and regulatory review.
- **Change and drift management**, recognizing that AI behavior may evolve due to data shifts, system updates, or regulatory changes.

Evaluation metrics should reflect operational stability and trustworthiness, not static performance snapshots.

Human-centered and workflow-aware oversight:

In high-volume automated environments, effective human oversight is best achieved through control-based design, including:

- **Role-based access and identity verification** tied to accountable oversight functions
- **Exception-based review**, directing human intervention to anomalous or high-risk scenarios
- **Clear accountability mapping** with defined ownership, oversight, and escalation responsibilities**,** ensuring accountable human authority for AI-enabled processes without requiring manual review of each AI-influenced transaction

This approach supports scalable oversight without degrading access, timeliness, or system performance.

Role of HHS:

HHS can support effective AI evaluation by:

- Encouraging **standardized, outcome-oriented evaluation principles** applicable to non-medical device AI
- Supporting **shared testing resources and reference workflows** that reflect real-world administrative and operational use
- Leveraging **existing federal mechanisms** (e.g., demonstrations, contracts, cooperative agreements) to pilot scalable evaluation and monitoring approaches
- Avoiding prescriptive testing mandates that duplicate existing security and compliance frameworks

Summary:

For non-medical device AI, the most effective evaluation approaches prioritize continuous monitoring, auditability, governance, and operational resilience. HHS engagement should reinforce scalable, standards-aligned evaluation practices that reflect the realities of enterprise healthcare infrastructure.

Question 5: How can HHS best support private sector activities (*e.g.,* accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS can best support private sector accreditation, certification, and credentialing by promoting **nationally consistent, standards-aligned assurance models** for AI used in healthcare, including **non-medical device AI** supporting clinical, administrative, and organizational functions. Such support is critical to **reducing redundant audits, unnecessary administrative burden, and escalating compliance costs**, while preserving strong security, privacy, and accountability safeguards.

Key areas HHS can strengthen private-sector assurance include:

- **Endorsing nationally recognized, NIST-aligned AI governance and security frameworks** that can serve as a common baseline for accreditation and certification, enabling organizations to demonstrate compliance against a consistent set of controls
- **Encouraging "audit once, rely many times" assurance models**, where independent, standards-based assessments are reusable across payers, providers, and other counterparties, rather than requiring repeated, duplicative audits
- **Supporting credentialing and qualification standards for AI assessors and auditors**, ensuring evaluations are conducted by entities with appropriate expertise in security, privacy, governance, and enterprise-scale operations
- **Standardizing audit evidence expectations**, so certifications focus on verifiable controls—such as governance documentation, change management, logging, monitoring, and traceability—rather than bespoke or ad hoc questionnaires
- **Investing in industry-driven testing resources**, including reference workflows and testing tools that reflect real-world interoperability and operational environments
- **"Reciprocity Framework"** for certifications:
  HHS should lead the creation of a "Common Audit Framework." If a clearinghouse's AI infrastructure is certified by a NIST-aligned federal body, that certification should satisfy all state and payer-specific audit requirements, eliminating the "death by a thousand audits" currently stifling innovation.

Summary:
By reinforcing **standards-based, certifiable, and reusable assurance frameworks**, HHS can promote innovation while reducing unnecessary audit duplication, cost, and friction for healthcare infrastructure operators—supporting trust and accountability without fragmenting oversight across the ecosystem.

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Question 6: Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short?

- What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Clearinghouses do not deploy AI for clinical decision-making or direct patient care and therefore cannot assess clinical performance or outcomes of AI tools used by providers. However, clearinghouses do have direct experience with AI deployed in **administrative and operational workflows that support clinical care**, where performance expectations are tied to **accuracy, timeliness, and transaction integrity**.

In these contexts, AI-enabled automation has met expectations where it:

- Improves routing, validation, and exception handling for high-volume transactions
- Reduces manual rework and processing delays
- Supports consistent application of business and compliance rules

AI has fallen short where:

- Controls do not adequately confirm **human review or authorization when required**
- Auditability and traceability are insufficient to demonstrate that transactions were reviewed, approved, or corrected appropriately
- Automated decisions cannot be reliably explained or validated within downstream workflows
- Focus on the **"Black Box"** failure of explainability.
  AI has exceeded expectations in pattern recognition for fraud detection. However, it has fallen short in "Explainable AI" (XAI). When a claim is flagged by AI, the lack of a "Reason Code" that a human can interpret leads to manual rework, negating the efficiency gains of the automation.

From an infrastructure perspective, the most promising AI capabilities are those that:

- Enhance **transaction-level validation and audit evidence**, including indicators that required human review has occurred
- Improve **interoperability, data quality, and exception detection** across multi-stakeholder workflows
- Reduce administrative friction that delays access to care, without introducing new risk or opacity

These uses do not replace clinical judgment but can materially improve the **efficiency, reliability, and cost-effectiveness** of the systems that support clinical care delivery.

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Question 7: Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care?

- What are the primary administrative hurdles to the adoption of AI in clinical care?

Clearinghouses do not make decisions regarding the adoption of AI for clinical decision-making or patient care. However, from an enterprise infrastructure and compliance perspective, adoption of AI that supports or interfaces with clinical care is most influenced by **organizational leadership responsible for risk, security, compliance, and operations**, rather than by clinical stakeholders alone.

In practice, the most influential roles include:

- **Executive leadership** responsible for enterprise risk and strategic alignment: Identify the **"CFO-CIO Alignment"** as the primary driver.
  Response Expansion: A primary hurdle is often the ROI Horizon. Because AI infrastructure requires high upfront costs for "clean data" pipelines, but the benefits (reduced friction) are often realized by other parties in the ecosystem, the "split incentive" prevents CIOs from securing funding.
- **Compliance, privacy, and security leadership**, who assess regulatory exposure and control readiness
- **IT and operational leadership**, who must integrate AI into existing workflows, data exchange, and transaction infrastructure

Even where there is interest in AI adoption, progress is often constrained by **administrative and governance hurdles**, including:

- **Unclear or inconsistent regulatory expectations**, particularly across federal and state jurisdictions
- **Lack of standardized governance and assurance frameworks**, leading to duplicative reviews and audits
- **Uncertainty around accountability and liability** across multi-stakeholder workflows
- **Integration and interoperability challenges**, especially where AI-enabled processes must align with existing transactions and compliance requirements

From a clearinghouse perspective, these administrative hurdles—not lack of innovation—are often the primary factors slowing responsible AI adoption at scale.

Question 8: Where would enhance interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care?

- Please consider specific data types, data standards, and benchmarking tools.

From a clearinghouse infrastructure perspective, enhanced interoperability would most meaningfully accelerate AI development by improving the **consistency, quality, and usability of administrative and operational data that supports clinical care**, rather than by expanding access to new clinical datasets alone. Clearinghouses operate at the intersection of multi-stakeholder data exchange and are uniquely positioned to observe where fragmentation limits AI scalability and trust.

The most impactful opportunities include:

- **Administrative and transactional data standardization:**
  Greater consistency in eligibility, prior authorization, claims, attachments, and payment-related data would enable AI tools to operate more reliably across organizations and jurisdictions. Variability in data formats, codes, and business rules currently limits the ability to train, evaluate, and deploy AI at scale.
- **Alignment and stability of data standards:**
  Continued alignment across standards such as X12, HL7/FHIR (where applicable), and related implementation guides would reduce translation complexity and enable AI systems to function predictably across interoperable environments.
- **Improved metadata and provenance indicators:**
  Standardized indicators for data origin, versioning, timing, and required human review or authorization would strengthen auditability and enable AI systems to distinguish between automated and human-validated inputs.
- **Benchmarking and reference datasets for operational use cases:**
  Industry-supported benchmarking tools, synthetic datasets, and reference workflows reflecting real-world administrative processes would help evaluate AI performance in operational contexts without introducing privacy or security risk.
- **Interoperability-aware benchmarking:**
  AI evaluation and benchmarking should reflect **multi-stakeholder workflows**, not isolated organizational environments, recognizing that administrative AI often spans providers, payers, and intermediaries.
- **Semantic Interoperability:** It is not enough to move data; the AI must *understand* it. We need standardized "Administrative Knowledge Graphs". If every payer uses a different definition for "medical necessity," AI cannot scale. We need HHS to mandate a national digital dictionary for administrative logic.

Enhanced interoperability in these areas would widen market opportunities for AI that improves **efficiency, transparency, and reliability** across healthcare operations, indirectly supporting access to care and cost reduction while preserving appropriate governance and accountability.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care?

- Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Clearinghouses do not engage directly with patients or caregivers and do not deploy AI for clinical decision-making. However, from an infrastructure and administrative perspective, clearinghouses observe systemic challenges that directly affect patient and caregiver experience, particularly where administrative processes support access to clinical care.

From this vantage point, patients and caregivers would reasonably expect AI to help address:

- **Delays in access to care** caused by administrative processes such as eligibility verification, prior authorization, and claims handling
- **Inconsistent or incorrect administrative outcomes** that result in rework, confusion, or unexpected financial responsibility
- **Lack of clarity around administrative decisions**, including how issues are resolved or corrected
- **Algorithmic Denial:** Patients fear that AI will be used to systematically deny care to increase margins. Our response should emphasize that clearinghouse AI is designed for **"Administrative Acceleration,"** not "Clinical Restriction," and that human-in-the-loop (HITL) overrides are hard-coded into the transaction path.

At the same time, patients and caregivers have well-founded concerns regarding:

- **Privacy and security of their health information**, especially as AI increases automation and data aggregation
- **Lack of transparency in automated decisions**, including difficulty understanding how outcomes are generated or how errors can be challenged
- **Accountability and recourse**, particularly whether meaningful human oversight exists when automated processes affect access, coverage, or payment

From a clearinghouse perspective, responsible use of AI should improve efficiency and timeliness while reinforcing **trust, transparency, and accountability** in the administrative processes that surround and support clinical care.

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Question 10: Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

a. Are there published findings about the impact of adopted AI tools and their use clinical care?

b. How does literature approach the costs, benefits, and transfers of using AI as part of clinical care?

Clearinghouses do not conduct or evaluate AI for direct clinical decision-making. However, from an enterprise compliance, security, and national healthcare infrastructure perspective, HHS can meaningfully accelerate responsible AI adoption by prioritizing research that addresses **real-world deployment, governance, and operational scalability**, including AI used in **non-medical device contexts** that support clinical care through administrative and organizational functions.

Priority areas for AI research:

- **Real-world performance and durability at scale:**
  Research on post-deployment AI behavior in production environments, including performance drift, error propagation, system updates, and long-term operational impacts—beyond controlled pilots or single-site studies.
- **Governance, accountability, and auditability mechanisms:**
  Practical research on how accountability is established and maintained in automated environments, including traceability, logging, exception-based escalation, and clearly defined ownership—without reliance on manual review of high-volume workflows.
- **Interoperability and data quality foundations:**
  Research on how standardized data formats, provenance indicators, and consistent implementation of interoperability standards improve AI reliability and comparability across multi-stakeholder environments, including the impact of fragmented state and federal requirements.
- **Security, privacy, and risk management for non-medical device AI:**
  Research on effective control frameworks for AI-enabled automation, including aggregation and monitoring, aligned with recognized standards (e.g., NIST-based approaches), and what constitutes sufficient, auditable assurance in practice.
- **Administrative and operational AI impacts that support clinical care:**
  Research on how AI affects timeliness, accuracy, and reliability of administrative processes that surround care delivery (e.g., authorization workflows, documentation and attachments, transaction validation), using measurable operational outcomes such as cycle time, error rates, rework, and dispute resolution.

- **Longitudinal Model Integrity:** Most research focuses on the *launch* of AI. HHS should fund research on **"Model Decay"**, how AI performance degrades over years as medical codes and payer policies shift. We need a "Post-Market Surveillance" equivalent for administrative algorithms to ensure they remain fair and accurate over time.

a) Published findings on impact

HHS should prioritize evidence that evaluates **real-world outcomes**, distinguishing between:

- results observed in controlled or pilot environments, and
- sustained performance across interoperable, multi-stakeholder production settings.

Consistent metrics and methodologies are critical to assessing whether AI delivers durable value beyond early adoption phases.

b) Costs, benefits, and transfers across the ecosystem

HHS should encourage research and literature that explicitly examines **how costs, benefits, and risks shift across participants**, including:

- **AI developers**, who may realize efficiency gains at the model level
- **Providers and payers**, who may experience workflow changes and operational impacts
- **Infrastructure operators, such as clearinghouses**, often absorb ongoing costs associated with integration, security controls, monitoring, audit support, and regulatory compliance

Such research should assess:

- the cumulative cost of maintaining AI-enabled infrastructure over time,
- the impact of duplicative audits and inconsistent oversight expectations, and
- where accountability and liability effectively reside in multi-stakeholder AI-enabled workflows.

Understanding these transfers is essential to ensure that AI adoption improves efficiency without unintentionally shifting disproportionate operational or compliance burden onto intermediaries that support national-scale healthcare transactions.

Summary of Question 10:

To accelerate responsible AI adoption, HHS should prioritize research that produces **deployable, scalable guidance** on governance, interoperability, security, and real-world operational impact—while explicitly accounting for **cost distribution and risk transfer** across developers, providers, payers, and healthcare infrastructure operators such as clearinghouses.

The Cooperative Exchange would be pleased to serve as an industry resource to CMS in support of government and private sector collaborative initiatives resulting in harmonious, national, scalable AI regulatory framework across the healthcare ecosystem.

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